

LEGISLATURE OF THE STATE OF IDAHO
Sixty-eighth Legislature Second Regular Session - 2026

IN THE HOUSE OF REPRESENTATIVES

HOUSE BILL NO. 863

BY HEALTH AND WELFARE COMMITTEE

AN ACT

RELATING TO PUBLIC ASSISTANCE; PROVIDING LEGISLATIVE FINDINGS AND INTENT;
AMENDING SECTION 56-265, IDAHO CODE, TO REVISE PROVISIONS REGARDING
PROVIDER PAYMENT; PROVIDING THAT CERTAIN ADMINISTRATIVE RULES CON-
TAINED IN IDAPA 16.03.26 SHALL BE NULL, VOID, AND OF NO FORCE AND EFFECT;
AND DECLARING AN EMERGENCY.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. LEGISLATIVE FINDINGS AND INTENT. The Legislature finds that providing oversight of Medicaid rates paid to residential habilitation providers and increasing transparency for taxpayers is one of the most important functions the Legislature provides. Currently, the state pays more than \$176.5 million from the general fund through Medicaid to help people with disabilities live independently with residential habilitation and other home and community-based services. In 2022, the Legislature approved a line item for \$70,393,100 to increase provider rates for adult developmental disabilities in residential habilitation settings in Chapter 252, Laws of 2022. Those provider rate increases were related to a line item of \$66,663,200 providing service enhancements and a new budget tool stemming from the *KW v. Armstrong* lawsuit. A new court order halted the implementation of that budget tool, and as a result, the service enhancements have not gone into effect. The state general fund match for the rate increases related to the service enhancement is approximately \$21,800,000 and can be reduced from the current budget. To support accountability and transparency for taxpayers and improve legislative oversight of provider payments, this statute change is necessary.

SECTION 2. That Section 56-265, Idaho Code, be, and the same is hereby amended to read as follows:

56-265. PROVIDER PAYMENT. (1) Where there is an equivalent, the payment to medicaid providers:

(a) May be up to but shall not exceed one hundred percent (100%) of the current medicare rate for primary care procedure codes as defined by the centers for medicare and medicaid services; and

(b) Shall be ninety percent (90%) of the current medicare rate for all other procedure codes.

~~(2) Where there is no medicare equivalent, the payment rate to medicaid providers shall be prescribed by rule. Where there is no medicare equivalent, the department may promulgate rules, subject to legislative approval, for payment rates. Residential habilitation, personal care services, developmental disability agency services, community-supported employment, and targeted service coordination shall be cost-surveyed annually with fifteen percent (15%) or more of responses being audited. The department shall~~

1 use information from the cost surveys and other sources to develop payment
2 rates, subject to legislative appropriation. Payment rates shall be devel-
3 oped to include allocations to direct care worker wages, employee-related
4 expenses, program-related expenses, and general and administrative costs.

5 (a) Providers are required on an annual basis to expend at least the
6 appropriated amount allocated to direct care workers and employee-re-
7 lated expenses to these categories.

8 (b) Failure of the provider to meet the requirement in paragraph (a) of
9 this subsection may result in a department-approved corrective action
10 plan, closure of intake, or termination of the provider agreement.

11 (c) The department shall summarize the required cost survey audits in
12 a publicly available report no later than December 31 of each calendar
13 year, with the first report being delivered by December 31, 2027.

14 (3) Notwithstanding any other provision of this chapter, if the
15 services are provided by a private, freestanding mental health hospital
16 facility that is an institution for mental disease as defined in 42 U.S.C.
17 1396d(i), the department shall reimburse for inpatient services at a rate
18 not to exceed ninety-one percent (91%) of the current medicare rate within
19 federally allowed reimbursement under the medicaid program. The reimburse-
20 ment provided for in this subsection shall be effective until July 1, 2021.

21 (4) The department shall, through the annual budget process, include
22 a line-item request for adjustments to provider rates. All changes to
23 provider payment rates shall be subject to approval of the legislature by
24 appropriation.

25 (5) Notwithstanding any other provision of this chapter, the depart-
26 ment may enter into agreements with providers to pay for services based on
27 their value in terms of measurable health care quality and positive impacts
28 to participant health.

29 (a) Any such agreement shall be designed to be cost-neutral or cost-
30 saving compared to other payment methodologies.

31 (b) The department is authorized to pursue waiver agreements with the
32 federal government as needed to support value-based payment arrange-
33 ments, up to and including fully capitated provider-based managed care.

34 (c) Beginning with the 2024 performance period and for all future per-
35 formance periods thereafter, federally qualified health centers and
36 any organization owned and controlled by a federally qualified health
37 center shall be exempt from any financial risk in value-based payment
38 agreements created pursuant to this section.

39 (6) Medicaid reimbursement for critical access, out-of-state, and
40 state-owned hospitals shall be as follows:

41 (a) In-state, critical access hospitals as designated according to 42
42 U.S.C. 1395i-4(c) (2) (B) shall be reimbursed at one hundred one percent
43 (101%) of cost;

44 (b) Out-of-state hospitals shall be reimbursed at eighty-seven percent
45 (87%) of cost;

46 (c) State-owned hospitals shall be reimbursed at one hundred percent
47 (100%) of cost; and

48 (d) Out-of-state hospital institutions for mental disease as defined
49 in 42 U.S.C. 1396d(i) shall be reimbursed at a per diem equivalent to
50 ninety-five percent (95%) of cost.

1 (7) The department shall equitably reduce net reimbursements for all
2 hospital services, including in-state institutions for mental disease but
3 excluding all hospitals and institutions described in subsection (6) of
4 this section, by amounts targeted to reduce general fund needs for hospital
5 payments by three million one hundred thousand dollars (\$3,100,000) in state
6 fiscal year 2020 and eight million seven hundred twenty thousand dollars
7 (\$8,720,000) in state fiscal year 2021.

8 (8) The department shall work with all Idaho hospitals, including in-
9 stitutions for mental disease as defined in 42 U.S.C. 1396d(i), to establish
10 value-based payment methods for inpatient and outpatient hospital services
11 to replace existing cost-based reimbursement methods for in-state hospi-
12 tals, other than those hospitals and institutions described in subsection
13 (6) of this section, effective July 1, 2021. Budgets for hospital payments
14 shall be subject to prospective legislative approval.

15 (9) The department shall work with Idaho hospitals to establish a
16 quality payment program for inpatient and outpatient adjustment payments
17 described in section 56-1406, Idaho Code. Inpatient and outpatient adjust-
18 ment payments shall be subject to increase or reduction based on hospital
19 service quality measures established by the department in consultation with
20 Idaho hospitals.

21 SECTION 3. The rules contained in IDAPA 16.03.26, Department of Health
22 and Welfare, relating to Medicaid Plan Benefits, Section 051.; and Section
23 052., shall be null, void, and of no force and effect on and after July 1,
24 2026.

25 SECTION 4. An emergency existing therefor, which emergency is hereby
26 declared to exist, this act shall be in full force and effect on and after its
27 passage and approval.